

## Prior Authorization Request - Inpatient Admission

### Health Plan

Humana Medicare Advantage. Plan ID: H5216-014-000. Request type: standard pre-service review.  
Date of request: 2026-10-04

### Member Information

Member: Loretta Vasquez-Moyer DOB: 1956-07-22 (age 70) Member ID: MBR-30458  
Address: 47 Sycamore Court, Canton, OH 44718  
Phone: (330) 555-0188 Sex: F  
Primary insurance: Humana Medicare Advantage (primary). Secondary: none on file.

### Requesting Provider

Dr. Samuel Whitcomb, MD -- Cardiac Electrophysiology  
NPI: 1649205738  
Practice: Stark County Heart & Rhythm Clinic  
Phone: (330) 555-0210 Fax: (330) 555-0211

### Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)  
CPT 33249 ICD-10: I42.0 (Dilated cardiomyopathy); I50.22  
Planned admit date: 2026-11-12  
Level of care requested: INPATIENT  
Place of service: Hospital, inpatient (acute care).

### Submission Contact

Submitted through the provider portal by Dana Kessler, Authorization Coordinator.  
Callback: (330) 555-0210. Supporting clinical documentation attached (7 pages including this request).

## History and Physical - Electrophysiology Clinic

Date: 2026-09-23. Patient: Loretta Vasquez-Moyer Age: 70 Sex: F  
Referring: Dr. D. Ferreira, cardiology.

### Chief complaint

Dilated cardiomyopathy with reduced EF, referred for evaluation of an implantable defibrillator.

### HPI

Ms. Vasquez-Moyer is a 70-year-old woman with a dilated cardiomyopathy and chronic systolic heart failure. She gets mildly short of breath climbing two flights of stairs or walking uphill, but is able to do her housework and shop for groceries without stopping. No orthopnea, no PND. Occasional ankle puffiness in summer heat. No syncope, no near-syncope, no palpitations. Functional status corresponds to NYHA class II.

### Cardiac history

Coronary CT angiography performed this summer showed no coronary stenosis, supporting a non-ischemic cause of the cardiomyopathy (see page 4). She has not had a heart attack. No stents, no bypass surgery. No recent MI, PCI or CABG. No history of ventricular arrhythmia.

### Other history

Hypertension. Type 2 diabetes, diet-controlled. Hypothyroidism. Osteoarthritis of both knees. Cholecystectomy 1998. Never smoker. Lives with husband, independent in ADLs. Retired schoolteacher.

### Exam

BP 126/74, HR 68, SpO2 98%, BMI 27. Neck veins not distended. Lungs clear. Regular rhythm, S3 absent, soft systolic murmur at apex. No peripheral edema. Neuro non-focal.

### Assessment

Symptomatic non-ischemic dilated cardiomyopathy, NYHA II, LVEF about 30%. Clinically stable at last visit.  
Plan: ICD implantation for primary prevention. Will request authorization for elective inpatient admission.

## Echocardiography Report

Date of study: 2026-08-26. Location: Stark County Heart & Rhythm Clinic, Echo Lab.

Patient: Loretta Vasquez-Moyer DOB: 1956-07-22 Reading cardiologist: V. Raman, MD.

### Summary of findings

Technically adequate study. The left ventricle is dilated (LVEDD 6.0 cm). Left ventricular systolic function is moderately to severely reduced, with an estimated LVEF of 30% (biplane Simpson). Diffuse hypokinesis. No LV thrombus. LA mildly dilated. RV size normal, systolic function low-normal. Mild functional MR, trace TR, RVSP 32 mmHg. No pericardial effusion. Aortic root normal.

### Conclusion

Dilated cardiomyopathy; LVEF approximately 30% by transthoracic echocardiography.

## Coronary CT Angiography Report

Date: 2026-07-14. Indication: new cardiomyopathy, exclude obstructive coronary artery disease.

Technique: prospectively gated, 128-slice scanner, 1 sublingual nitroglycerin, 75 mL contrast. HR 58.

### Findings

Coronary calcium score 0 (Agatston). No coronary plaque. All three major coronary arteries and branches are patent without stenosis. Left main, LAD, LCx and RCA are normal. Right dominant circulation.

Cardiac chambers: dilated left ventricle. No intracardiac thrombus. Pulmonary veins normal. Incidental 4 mm noncalcified RUL nodule, low-risk, no follow-up required in a never-smoker.

### Impression

No coronary artery disease. Non-ischemic dilated cardiomyopathy.

## Current Medication List

Patient: Loretta Vasquez-Moyer Printed from EHR 2026-09-23. Allergies: penicillin (hives).

### Active outpatient medications

sacubitril/valsartan 97/103 mg tablet, one tablet twice daily

metoprolol succinate 100 mg tablet, once daily

spironolactone 25 mg tablet, once daily

empagliflozin 10 mg tablet, once daily

furosemide 20 mg tablet, once daily as needed for swelling

levothyroxine 75 mcg tablet, once daily before breakfast

acetaminophen 500 mg tablet, as needed for knee pain

calcium with vitamin D, once daily

Pharmacy: Giant Eagle Pharmacy, Canton. Med list reviewed with patient at visit, no changes today.

Immunizations: influenza 2025, COVID-19 2025, pneumococcal PCV20 2024.

## ECG Report and Laboratory Results

### 12-lead ECG, 2026-09-23

Normal sinus rhythm at 66 bpm. PR 168 ms. QRS 98 ms, narrow. QTc 438 ms. No ST-T changes. No evidence of prior myocardial infarction. No arrhythmia on the tracing. Interpreted by: B. Lindgren, MD.

### Laboratory results, 2026-09-23

| Test       | Result  | Reference                     |
|------------|---------|-------------------------------|
| Sodium     | 140     | 135-145 mmol/L                |
| Potassium  | 4.3     | 3.5-5.0 mmol/L                |
| Creatinine | 0.9     | 0.6-1.2 mg/dL                 |
| eGFR       | 68      | >60 mL/min/1.73m <sup>2</sup> |
| NT-proBNP  | 780 (H) | <450 pg/mL                    |
| Hemoglobin | 12.8    | 12.0-15.5 g/dL                |
| HbA1c      | 6.2%    |                               |
| TSH        | 2.4     | 0.4-4.5 mIU/L                 |

## Electrophysiology Surgeon Operative Plan

Date: 2026-09-25. Surgeon: Dr. Samuel Whitcomb, MD, cardiac electrophysiology. Patient: Loretta Vasquez-Moyer.

### Procedure

Transvenous ICD implantation (single chamber), left pectoral pocket, CPT 33249.

### Indication

Primary prevention in non-ischemic dilated cardiomyopathy, LVEF 30%, NYHA class II.

### Expected stay

Two midnights of inpatient care are expected: overnight telemetry, post-implant chest radiograph, device interrogation the next morning, and observation of the pocket. Discharge on the second morning, assuming no complications.

### Comorbidities and risk factors

Heart failure with reduced EF, hypertension, type 2 diabetes (diet-controlled), hypothyroidism.

### Clinical status

Clinically stable and euvolemic. No limiting conditions: no severe neurologic injury, no non-cardiac illness with a prognosis under 1 year, rhythm is sinus (no atrial fibrillation). Consent will be obtained on the day of admission.